

D548

Emergency Management and Planning in Healthcare

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

Method © 2026 Erik Daniel Lindquist — Course Mastery Coach, armored drill. Items drafted with AI assistance under his direction.

How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word _____	The one fact that decides _____	Answer _____	[] sure [] unsure [] guess
-----------------------------	---	------------------------	----------------------------------

Items

1.

After a 2019 flood, a hospital that serves three rural counties moves its backup generators and main electrical panels from the basement to the second floor. The project was paid for with a state resilience grant. Which emergency management phase does this work belong to?

- A. Recovery.
- B. Response, because it was triggered by the 2019 flood.
- C. Mitigation.
- D. Preparedness.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

A hospital runs a full-scale mass-casualty drill with the local fire department and restocks its disaster supply cache. Which phase is this?

- A. Preparedness.
- B. Recovery.
- C. Mitigation.
- D. Response.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A medical center's HVA team scores each hazard's probability and impact from 1 to 4, then multiplies them. Hurricane: probability 2, impact 4. Winter storm: probability 4, impact 1. Ransomware attack: probability 3, impact 3. Chemical spill: probability 1, impact 3. The center's last hurricane made national news. Which hazard should be the top planning priority?

- A. Chemical spill.
- B. Ransomware attack.
- C. Hurricane.
- D. Winter storm.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4. [TRANSFER — reversed stem / different variable]

In an HVA that uses risk = probability × impact, a hazard has a risk score of 12 and a probability score of 3. What is its impact score?

- A.** 9
- B.** 12
- C.** 4
- D.** 36

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

During a surge after a train derailment, the hospital incident commander, a veteran ED physician who helped write the emergency plan, finds that 11 unit leaders are reporting directly to her. What change best fits incident command principles?

- A.** Have each unit leader report to two section chiefs so nothing is missed.
- B.** Replace her, because the person who wrote the plan should not run it.
- C.** Keep all 11 reports, because the commander must personally direct every unit.
- D.** Activate section chiefs (operations, planning, logistics, finance) so she has about five direct reports.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6. [TRANSFER — reversed stem / different variable]

A logistics unit leader receives conflicting orders from both the operations chief and the planning chief. Which ICS principle is being violated?

- A.** Span of control.
- B.** Unity of command.
- C.** Common terminology.
- D.** Modular organization.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

Two hours after a water main break shuts down a hospital's dialysis unit, reporters are calling. The repair time is still unknown. The hospital's public relations director won an industry award last year. What should the first public message do?

- A. Decline comment and refer all questions to the city water utility.
- B. Share what is known now, say what is not yet known and when the next update will come, and tell dialysis patients where to go.
- C. Wait until the repair time is confirmed so nothing has to be corrected later.
- D. Promise that service will be restored within the hour to calm patients.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

At a bus crash, a responder using START finds an adult who cannot walk, is breathing 34 times a minute, and has a deep cut on his forearm. He says he is a retired firefighter and is calm. Which category applies?

- A. Delayed (yellow).
- B. Minor (green), because he is calm and talking.
- C. Expectant (black).
- D. Immediate (red).

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9. [TRANSFER — reversed stem / different variable]

Under START, a patient is not breathing, and breathing does not start after the airway is repositioned. Which category applies?

- A. Immediate (red).
- B. Immediate, because every non-breathing patient always gets CPR first.
- C. Expectant/deceased (black).
- D. Delayed (yellow).

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A 400-bed hospital is running at 85% occupancy. Its surge plan says it can reach 120% of licensed beds in a crisis by using hallways and recovery rooms. The CEO recently joined the state hospital association board. How many additional patients can it absorb at full surge?

- A. 140
- B. 60
- C. 480
- D. 80

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

The day after a mass shooting, several ED nurses who treated victims seem shaken. The hospital, which has a new wellness app, wants to support them. The nurse manager is known for a direct style. Which approach best follows psychological first aid principles?

- A. Email everyone a link to the wellness app and consider support provided.
- B. Require every nurse to attend a group session and recount the event in detail.
- C. Wait for nurses to ask for help, since most people recover on their own.
- D. Check in with each nurse, attend to practical needs, listen without pressing for details, and link those who want it to counseling or the EAP.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12.

During a regional respiratory surge, a hospital has 10 ventilators and 16 patients who need one. The state has activated crisis standards of care. One patient is a major donor to the hospital's foundation. How should ventilators be allocated?

- A. By a transparent, pre-set triage protocol based on likely short-term survival, applied equally and run by a triage team separate from bedside clinicians.
- B. Give the donor priority, since the foundation funds the ICU.
- C. Let each bedside physician decide for their own patients.
- D. First come, first served, as in normal operations.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer C Concept 1

After a 2019 flood, a hospital ~~that serves three rural counties~~ moves its backup generators and main electrical panels from the basement to the second floor. ~~The project was paid for with a state resilience grant.~~ Which emergency management phase does this work belong to?

Target word: name the phase **Decides:** Relocating equipment reduces the impact of future floods.

A	Adjacent category	Recovery restores operations after an event; this work reduces the next one's damage.
B	Camouflage	The old flood prompted the project; response is action during an event.
C	Correct	Permanent changes that lessen a hazard's future impact are mitigation.
D	Confusion set	Preparedness is plans, training, drills and supplies, not structural change.

2. Answer A Concept 1 · transfer

A hospital runs a full-scale mass-casualty drill with the local fire department and restocks its disaster supply cache. Which phase is this?

Target word: name the phase — different activity **Decides:** Drills and stockpiles build readiness.

A	Correct	Training, exercises and stockpiling are preparedness.
B	Inverted operation	Recovery is the far end of the cycle, after an event.
C	Confusion set	Mitigation changes the hazard's impact; drills do not.
D	Adjacent category	A drill simulates response; no real event is under way.

3. Answer B Concept 2

A medical center's HVA team scores each hazard's probability and impact from 1 to 4, then multiplies them. Hurricane: probability 2, impact 4. Winter storm: probability 4, impact 1. Ransomware attack: probability 3, impact 3. Chemical spill: probability 1, impact 3. ~~The center's last hurricane made national news.~~ Which hazard should be the top planning priority?

Target word: highest risk score **Decides:** Scores: hurricane 8, winter storm 4, ransomware 9, chemical spill 3.

A	Inverted operation	Picks the lowest score; lower risk means lower priority.
B	Correct	$3 \times 3 = 9$, the highest risk score.
C	Camouflage	Highest impact and memorable, but $2 \times 4 = 8$ is below 9.
D	Dropped element	Ranks by probability alone and ignores impact.

4. Answer C Concept 2 · transfer

In an HVA that uses $\text{risk} = \text{probability} \times \text{impact}$, a hazard has a risk score of 12 and a probability score of 3. What is its impact score?

Target word: solve for impact **Decides:** $\text{Impact} = 12 / 3$.

A	Adjacent category	Subtracts as if the scores were added together.
B	Wrong stem number	That is the risk score, not impact.
C	Correct	$12 / 3 = 4$.
D	Inverted operation	Multiplies instead of dividing.

5. Answer D Concept 3

During a surge after a train derailment, the hospital incident commander, ~~a veteran ED physician who helped write the emergency plan~~, finds that 11 unit leaders are reporting directly to her. What change best fits incident command principles?

Target word: fix the span of control **Decides:** 11 direct reports is far above the 3–7 range.

A	Confusion set	Violates unity of command; each person should have one supervisor.
B	Camouflage	Her role in writing the plan is irrelevant; the structure is the problem.
C	Absolute language	ICS delegates through sections precisely so the commander is not overloaded.
D	Correct	Adding a layer restores a manageable span of control.

6. Answer B Concept 3 · transfer

A logistics unit leader receives conflicting orders from both the operations chief and the planning chief. Which ICS principle is being violated?

Target word: name the principle from the symptom **Decides:** Two supervisors giving orders to one person.

A	Confusion set	Span of control is how many report to one leader, not how many leaders one person has.
B	Correct	Each person should report to only one supervisor.
C	Adjacent category	Shared terms prevent confusion but do not address dual reporting.
D	Adjacent category	Modularity is expanding or shrinking the structure to fit the incident.

7. Answer B Concept 4

Two hours after a water main break shuts down a hospital's dialysis unit, reporters are calling. The repair time is still unknown. ~~The hospital's public relations director won an industry award last year.~~ What should the first public message do?

Target word: content of the first message **Decides:** Facts are incomplete but the public needs guidance now.

A	Adjacent category	The utility owns the pipe, but the hospital owns its patients' care messages.
B	Correct	Be first, be right, be credible, and give protective actions.
C	Inverted operation	Silence cedes the story to rumor; speed matters in a crisis.
D	Absolute language	Over-promising on an unknown destroys credibility when it fails.

8. Answer D Concept 5

At a bus crash, a responder using START finds an adult who cannot walk, is breathing 34 times a minute, and has a deep cut on his forearm. ~~He says he is a retired firefighter and is calm.~~ Which category applies?

Target word: START category **Decides:** Respiratory rate over 30.

A	Dropped element	Skips the respiratory check that moves him to immediate.
B	Camouflage	Calm speech does not override the respiratory criterion, and he cannot walk.
C	Confusion set	Expectant is for patients not breathing after airway repositioning.
D	Correct	A respiratory rate above 30 is an immediate criterion in START.

9. Answer C Concept 5 • transfer

Under START, a patient is not breathing, and breathing does not start after the airway is repositioned. Which category applies?

Target word: START category — other end **Decides:** No breathing after airway repositioning.

A	Confusion set	Red applies if breathing starts after repositioning.
B	Absolute language	Mass-casualty triage does not start CPR; scarce hands go to savable patients.
C	Correct	No spontaneous breathing after repositioning is the expectant criterion.
D	Adjacent category	Delayed patients are breathing and following commands.

10. Answer A Concept 6

A 400-bed hospital is running at 85% occupancy. Its surge plan says it can reach 120% of licensed beds in a crisis by using hallways and recovery rooms. ~~The CEO recently joined the state hospital association board.~~ How many additional patients can it absorb at full surge?

Target word: added patients at surge **Decides:** Surge max 480 – current census 340.

A	Correct	$400 \times 1.20 = 480$; $400 \times 0.85 = 340$; $480 - 340 = 140$.
B	Adjacent category	Counts only empty licensed beds and ignores the surge space.
C	Wrong stem number	That is total surge capacity, not the added patients.
D	Dropped element	Counts only the surge beds above 400 and ignores the 60 empty licensed beds.

11. Answer D Concept 7

The day after a mass shooting, several ED nurses who treated victims seem shaken. The hospital, ~~which has a new wellness app,~~ wants to support them. ~~The nurse manager is known for a direct style.~~ Which approach best follows psychological first aid principles?

Target word: best support approach **Decides:** Voluntary, practical support and linkage.

A	Camouflage	The app may help, but it replaces the personal check-in PFA calls for.
B	Confusion set	Mandatory debriefing that forces retelling is not recommended and can cause harm.
C	Dropped element	Drops the proactive look-and-check step.
D	Correct	Look, listen, link, voluntarily, is the core of psychological first aid.

12. Answer A Concept 8

During a regional respiratory surge, a hospital has 10 ventilators and 16 patients who need one. The state has activated crisis standards of care. ~~One patient is a major donor to the hospital's foundation.~~ How should ventilators be allocated?

Target word: allocation method under crisis standards **Decides:** Crisis standards are active and demand exceeds supply.

A	Correct	Crisis standards shift to saving the most lives with a fair, consistent process.
B	Camouflage	Social or financial status must not affect allocation.
C	Dropped element	Drops the separate triage team and consistent rules that keep decisions fair.
D	Adjacent category	That is the conventional standard; crisis standards replace it when supply runs out.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	5	
Wrong stem number	A figure copied from the prompt that is not the answer.	2	
Adjacent category	The right idea in the wrong bucket.	9	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	4	
Absolute language	Always, never, guarantee — where judgment applies.	3	
Confusion set	Every option is real; the test is exact role-matching.	7	
Camouflage	Answers a stem fact that was never the question.	6	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	The four phases of emergency management	Mitigation reduces future impact; preparedness builds plans, training and stockpiles; response acts during the event; recovery restores operations afterward.	Calling any work done after a past disaster recovery, or calling mitigation preparedness.	Whether the action changes the hazard's impact, gets people ready, handles an event now, or rebuilds.
2	Hazard vulnerability analysis (HVA)	Rank hazards by risk = probability × impact (adjusted for preparedness), not by probability or impact alone.	Planning first for the most dramatic or most frequent hazard.	A table of hazards with separate probability and impact scores.
3	Incident command structure (HICS/ICS)	Keep span of control at about 3–7 direct reports (5 ideal) and give each person one supervisor (unity of command).	Having the commander run every unit personally, or having staff report to two chiefs.	A count of people reporting to one leader, or conflicting orders.
4	Crisis communication	Be first, be right, be credible: release what is known promptly, say what is not yet known, give protective actions and the time of the next update.	Waiting for complete facts, or over-promising to calm people.	Reporters calling while key facts are still unclear.
5	START triage	Walking = minor (green); not breathing after airway repositioning = expectant (black); respirations over 30, poor perfusion, or cannot follow commands = immediate (red); otherwise delayed (yellow).	Triaging by visible injuries or by how calm the patient seems.	A respiratory rate, a capillary refill time, or whether the patient can walk.
6	Surge capacity	Additional capacity = surge maximum – current census, using the plan's surge percentage of licensed beds.	Counting only empty licensed beds, or reporting the surge total as added capacity.	An occupancy percentage plus a surge percentage of licensed beds.
7	Psychological first aid	Offer voluntary, practical support: check needs, listen without pushing for details, and link people to further help.	Mandatory group debriefings that make everyone retell the event.	Staff or survivors shaken after a traumatic event.
8	Crisis standards of care	When resources are truly scarce, allocate by a transparent, pre-set protocol aimed at saving the most lives, applied equally and run by a triage team separate from bedside care.	First come, first served, or letting status or wealth decide.	More patients than ventilators, beds, or drugs, and an activated crisis declaration.

Course scope (public description): Emergency Management and Planning in Healthcare equips students with the necessary knowledge and skills to effectively manage emergencies in healthcare organizations. Students will learn to assess, plan, and respond to emergencies, ensuring the safety of patients and staff. This course covers evaluating the impact of emergencies, identifying vulnerabilities, and assessing risks, with a focus on crisis communication and ethical considerations specific to healthcare. Students will gain insights into leadership's role in decision-making, coordination, and resource allocation during critical situations, while also exploring public

health, community resilience, collaboration, and the integration of technology. Additionally, the course addresses the influence of cultural factors and principles of mental health support in enhancing emergency management capabilities.